Menstrual History — OSCE Checklist

Tick each item as you ask it. **Shaded rows (*)** *are asked if relevant.* † *= added from standard teaching, not in the Deakin guideline. Use sensitive, non-judgemental language and establish the patient's goals and concerns.*

1. Opening

☐	Introduce self, confirm patient ID, explain purpose; consent
☐	Signpost: "Some of these questions are personal — you don't have to answer anything you're not comfortable with."
☐	Open question: "Do you have periods or a menstrual cycle?"

2. The cycle

	Ask about	Example question	Notes
☐	Menarche	"How old were you when your periods started?"	
☐	Menopause*	"Have your periods stopped?"	Menopause = 12 months since the final period; before this = perimenopause
☐	LMP / LNMP	"When did your last normal period start?"	Any chance of pregnancy?
☐	Regularity	"Do they come at about the same time each month?"	Regular vs irregular
☐	Cycle length	"How many days from the start of one period to the start of the next?"	Normal ~21–35 days
☐	Duration	"How many days do you bleed for?"	Normal ~4–7 days (30–80 mL)
☐	Tracking	"Do you track your cycle, e.g. with an app?"	Helps with oligo-/amenorrhoea

3. Problems with periods

	Ask about	Example question	Notes
Heavy menstrual bleeding (HMB) — "Is it too heavy?"			
☐	Clots, flooding, leakage	"Do you pass clots or leak through?"	Suggests HMB
☐	Products	Pads, tampons, cups, period underwear; using two at once?	Dual products = heavy flow
☐	Frequency of change	"How often do you change, and are they soaked?"	Change <2-hourly = significant
☐	Impact	Physical, emotional, social, work/school	Defines HMB
☐	Anaemia symptoms*	Fatigue, breathlessness, dizziness †	Iron deficiency
Dysmenorrhoea — "Is it too painful?"			
☐	Timing and site	When does pain start (may precede bleeding by days)? Where? How long?	
☐	Impact	Missing school or work? What helps?	
☐	Associated symptoms	Nausea, vomiting, diarrhoea, constipation	Consider endometriosis
Cycle too frequent or infrequent			
☐	Oligo- / amenorrhoea	Cycle length; periods stopped? Pregnancy?	Consider PCOS, thyroid, weight change †
Abnormal bleeding — red flags			
☐	Intermenstrual (IMB)	Bleeding between periods?	Cervical / endometrial pathology
☐	Post-coital (PCB)	Bleeding after sex?	Cervical pathology
☐	Post-menopausal (PMB)	**Any** bleeding or spotting after menopause?	Red flag — exclude endometrial cancer urgently

4. Management of periods

☐	Hormonal	COCP, POP, IUD, implant, injection — not always used for contraception	All can alter the cycle
☐	Non-hormonal	NSAIDs, tranexamic acid	Pain and flow control

5. Link to related history and close

☐	Gynaecological, obstetric, sexual health (± gender identity) and surgical history as relevant
☐	Thyroid symptoms* (heat/cold intolerance, weight change) — thyroid disease can change menstrual bleeding
☐	Ideas, concerns, expectations: what does the patient want? (some wish to have, others not to have periods)
☐	Summarise back, check accuracy, invite questions, thank patient

6. Quick reference

Type	Causes of HMB
Structural	Fibroids (leiomyomata), adenomyosis, endometrial polyps, endometrial hyperplasia, endometrial cancer
Non-structural	Ovulatory disorders (PCOS, perimenopause, puberty), clotting disorders, iatrogenic (IUD), hypothyroidism
Mnemonic †	PALM-COEIN

Term	Meaning
HMB	Excess loss interfering with quality of life
IMB	Bleeding between periods
PCB	Bleeding after intercourse
PMB	Any bleeding ≥12 months after the final period
Cycle length	Day 1 of one period to day 1 of the next